

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the administration of Fusidic acid 2% cream and Flucloxacillin 250mg/5ml oral suspension for the treatment of Impetigo

Summary of NICE / NICE CKS Guidelines for Impetigo

Impetigo - NICE Guidance Overview

Impetigo is a highly contagious superficial bacterial skin infection, usually caused by *Staphylococcus aureus* or *Streptococcus pyogenes* (or both). It is common in children aged 2-5 years and can spread rapidly in settings with poor hygiene.

Clinical Presentation

- **Non-bullous impetigo (70-80% of cases):** Small vesicles or pustules that rapidly become honey-crusted erosions. Usually on exposed areas (face, limbs).
- **Bullous impetigo (20-30% of cases):** Fluid-filled blisters that rupture easily, leaving superficial erosions. Often in flexural areas.
- Lesions may be itchy or tender; regional lymphadenopathy is common.

Microbiology

- ***Staphylococcus aureus*:** Most common causative agent (including MRSA in some areas); resistant to penicillin V.
- ***Streptococcus pyogenes*:** Common co-pathogen; increasingly important cause.
- Local resistance patterns should guide antibiotic choice.

Treatment Approach

- Topical therapy: Fusidic acid cream for non-bullous and limited disease (localized lesions <5cm).
- Systemic therapy: Flucloxacillin for widespread, bullous, or failed topical treatment.
- Duration: 5-7 days of topical therapy; 5-7 days of systemic therapy.
- School exclusion: Until 48 hours after starting antibiotics or until lesions are crusted over (whichever is later).

Patient Group Direction

for the supply/administration of

Fusidic acid 2% cream

for the treatment of

Impetigo

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	- Pharmacist registered and practicing with the GPhC - Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. - All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines - All users must previously have used PGDs to supply medication - Must work in compliance with the SOPs of their own employer - All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	- All users must be up to date with CPD requirements and appraisal - All users must be up to date with MHRA and safety alerts with regards to medical products - All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of localized non-bullous impetigo and as first-line therapy for limited superficial skin infections
Inclusion criteria	Children and adults with impetigo Non-bullous impetigo with limited extent (typically <5cm or <3-5 lesions in localized area) Cases where systemic therapy is not indicated Parental/guardian consent (for children <16 years) Intact or minimally broken skin (fusidic acid penetration adequate for superficial infection)
Exclusion criteria	Known hypersensitivity to fusidic acid or excipients Bullous impetigo (use systemic therapy instead) Widespread impetigo (>5 lesions or lesions >5cm total; use systemic therapy) Lesions involving face with significant systemic symptoms Immunocompromised patients (generally require systemic therapy)
Cautions	Avoid prolonged use (>2 weeks) due to risk of resistance emergence. Do not apply to large body surface areas; systemic therapy preferred

	for extensive disease. May be less effective in MRSA colonisation; consider alternative if treatment failure. Avoid contact with eyes; if contact occurs, rinse thoroughly with water. Avoid occlusion for prolonged periods. Monitor for signs of treatment failure; if no improvement by 48 hours, switch to systemic therapy.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Fusidic acid 2% cream
Legal category	POM
Storage	Store at room temperature, protected from heat. Keep in original tube with cap tightly closed.
Route/method of administration	Topical - apply directly to affected skin areas
Dose and frequency	Apply a thin layer to affected area three times daily for 5-7 days Cover entire lesion and immediate surrounding area (about 1cm margin) Can be covered with a dressing if required
Quantity to be administered and/or supplied	1 tube of 15g
Maximum or minimum treatment period	5-7 days
Adverse effects	Local irritation at application site (mild, transient) Contact dermatitis or local hypersensitivity reaction Rarely, systemic hypersensitivity reactions

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP

	• name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Provide the patient information leaflet (PIL) supplied with the medication. Explain the importance of hygiene measures and completing the full course of antibiotics to prevent complications and transmission to others.
Follow-up advice to be given to patient or carer	Complete the full course of antibiotics as prescribed, even if symptoms improve within a few days. If using topical therapy: apply as directed to all affected areas three times daily; wash hands after application. If taking oral antibiotics: take exactly as prescribed, at regular intervals; preferably on an empty stomach. Strict hygiene measures are essential: keep the affected area clean and dry; avoid scratching or picking at lesions; keep nails short to reduce trauma; use separate towels and flannels (do not share); wash affected area and change dressings regularly. Avoid sharing: towels, flannels, bedding, clothing, toys, or personal items with others, especially other children. Handwashing: wash hands frequently, especially after touching the affected area. School/childcare exclusion: child should not attend school or nursery until 48 hours after starting antibiotics OR until all lesions are completely crusted over (whichever is later). Seek immediate medical attention if: signs of spreading infection develop, fever worsens or persists, signs of cellulitis or systemic infection appear, or allergic reactions occur. Return to GP if symptoms do not improve within 48 hours of starting antibiotics. For topical therapy: switch to oral antibiotics if lesions do not

	improve by 48 hours or if infection spreads.
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Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017
<https://www.nice.org.uk/guidance/mg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017
<https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Impetigo: NICE Clinical Knowledge Summaries
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
1/11/25	31/10/26		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
	(GPhC: 2046322)		
Name of professional group signatory	Job title & name of organisation	Signature	Date
	(GPhC: 2046322)		

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		2/11/25
Chris Pilkington	Pharmacist GPhC: 2046322		2/11/25

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction
for the supply/administration of
Flucloxacillin 250mg/5ml oral suspension
for the treatment of
Impetigo

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGDs to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
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PGD Indication	Treatment of bullous or widespread impetigo, or non-bullous impetigo unresponsive to topical therapy
Inclusion criteria	Children and adolescents aged 3 months to 18 years with impetigo Bullous impetigo (requires systemic therapy) Widespread non-bullous impetigo (>5 lesions or spread over multiple body areas) Failure of topical therapy after 48 hours Parental/guardian consent (for children <16 years) Ability to take oral medication No allergy to penicillin/flucloxacillin

Exclusion criteria	Known penicillin allergy (any type of reaction) or flucloxacillin hypersensitivity Previous cholestasis or jaundice associated with flucloxacillin use Cephalosporin allergy with high risk of cross-reactivity to penicillins Severe hepatic or renal impairment (eGFR <30 mL/min/1.73m²)
Cautions	Hepatic monitoring: Flucloxacillin can rarely cause hepatic reactions; monitor for signs of jaundice or hepatotoxicity. Gastrointestinal upset is common; take with or after food if GI symptoms occur (though absorption may be reduced). Drug interactions: May reduce effectiveness of oral contraceptives. Monitor for Clostridioides difficile-associated diarrhoea, especially in prolonged therapy. Use with caution in severe renal impairment; dose adjustment may be required. History of penicillin allergy: only use if allergy was non-severe (e.g., mild rash) and benefit outweighs risk; cross-reactivity with cephalosporins is approximately 1-3%.
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Flucloxacillin 250mg/5ml oral suspension
Legal category	POM
Storage	Store dry powder or tablets at room temperature. After reconstitution, refrigerate suspension at 2-8°C and discard after 14 days.
Route/method of administration	Oral
Dose and frequency	12.5 mg/kg four times daily for 5-7 days (standard therapy) 25 mg/kg four times daily for 5-7 days (for severe or disseminated infection) Doses should be calculated based on actual body weight Administer on an empty stomach (1 hour before or 2 hours after food) for optimal absorption
Quantity to be administered and/or supplied	Appropriate volume for age and weight to complete prescribed course
Maximum or minimum	5-7 days

treatment period	
Adverse effects	Gastrointestinal upset (nausea, vomiting, diarrhoea, abdominal pain) Rash and urticaria Candida infections (oral thrush, vaginal candidiasis) Hepatotoxicity (cholestasis, jaundice) - rare but serious Pseudomembranous colitis (associated with C. difficile) Allergic reactions including anaphylaxis (rare in non-allergic individuals) Stevens-Johnson syndrome (very rare)

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP - name of healthcare practitioner - name and brand of medication - date of supply dose, form and route quantity supplied - advice given, including advice given if excluded or declines treatment - details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: - Adults aged 18 years and over - keep records for 8 years - Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

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Follow-up advice to be given to patient or carer	Complete the full course of antibiotics as prescribed, even if symptoms improve within a few days. If using topical therapy: apply as directed to all affected areas three times daily; wash hands after application.

	If taking oral antibiotics: take exactly as prescribed, at regular intervals; preferably on an empty stomach. Strict hygiene measures are essential: keep the affected area clean and dry; avoid scratching or picking at lesions; keep nails short to reduce trauma; use separate towels and flannels (do not share); wash affected area and change dressings regularly. Avoid sharing: towels, flannels, bedding, clothing, toys, or personal items with others, especially other children. Handwashing: wash hands frequently, especially after touching the affected area. School/childcare exclusion: child should not attend school or nursery until 48 hours after starting antibiotics OR until all lesions are completely crusted over (whichever is later). Seek immediate medical attention if: signs of spreading infection develop, fever worsens or persists, signs of cellulitis or systemic infection appear, or allergic reactions occur. Return to GP if symptoms do not improve within 48 hours of starting antibiotics. For topical therapy: switch to oral antibiotics if lesions do not improve by 48 hours or if infection spreads.
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